



JAZAN HEALTH CLUSTER

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Stroke Public Education Plan

Community Stroke Awareness Program — Aligned with AHA/ASA Guidelines (2026)

Hospital / Cluster: _____

Stroke Program Director: _____

Stroke Program Coordinator: _____

Approved By: Stroke Interprofessional Committee (IPC)

Review Cycle: Annually

Document Control

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1.0	_____	Stroke Program Coordinator	Initial issue for 2026 community awareness cycle

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1. Purpose

To raise community awareness of stroke warning signs, risk factors, and the importance of immediate emergency response, in alignment with the American Heart Association / American Stroke Association (AHA/ASA) public education guidance and Target: Stroke best-practice strategies. Early recognition and rapid activation of emergency medical services (EMS) are the single greatest determinant of a patient's eligibility for time-sensitive stroke treatment.

2. Goals and Objectives

By the end of the annual cycle, the Stroke Public Education Plan aims to:

- Increase community recognition of stroke warning signs using the BE-FAST mnemonic (Balance, Eyes, Face, Arm, Speech, Time).
- Promote immediate activation of emergency medical services (EMS) rather than self-transport when stroke is suspected.
- Raise awareness of modifiable stroke risk factors (hypertension, diabetes, smoking, atrial fibrillation, high cholesterol, physical inactivity).
- Educate high-risk populations on preventive lifestyle measures and the importance of regular screening.
- Reduce community-level pre-hospital delay (onset-to-door time) for suspected stroke patients.
- Build sustained public-facing partnerships with schools, mosques, community centers, and media outlets.

3. Target Audience

- General public within the hospital catchment area.
- High-risk groups: patients with hypertension, diabetes, atrial fibrillation, or prior stroke/TIA.
- Older adults and their caregivers.
- School students and staff (basic awareness).
- Community and religious leaders (mosque, community center outreach).
- Local media and social media audiences.

4. Key Public Messages (AHA/ASA Aligned)

All public materials and outreach activities are built around the AHA/ASA BE-FAST framework:

- B — Balance: Sudden loss of balance or coordination.
- E — Eyes: Sudden blurred or double vision, or loss of vision.
- F — Face: Facial drooping, especially on one side.
- A — Arm: Arm weakness or numbness, especially on one side.
- S — Speech: Slurred speech or difficulty speaking/understanding.
- T — Time: Time to call emergency services immediately — every minute matters.

Messaging also reinforces that stroke can happen at any age, that most strokes are preventable through risk-factor control, and that calling EMS — not self-transport — gives patients the best chance of receiving time-sensitive treatment.

5. Educational Strategies and Channels

- Community health talks and awareness sessions at primary care centers.
- Health fairs and free blood-pressure / risk-factor screening events.
- BE-FAST awareness campaigns in shopping malls and public spaces.
- School-based awareness sessions for staff and older students.
- Mosque and community-center outreach sessions during high-attendance periods.
- Printed materials: brochures, posters, and BE-FAST wallet cards in Arabic and English.
- Digital and social media campaigns aligned with World Stroke Day (29 October).
- Local media appearances (radio, TV, press) featuring stroke specialists.

6. Annual Public Awareness Calendar

Month	Activity	Audience	Channel / Method
January	Launch of Annual Awareness Campaign	General public	Media / social media
February	Risk Factor Screening Drive	High-risk groups	Screening event
March	BE-FAST School Program	Students & staff	School sessions
April	Community Health Talk Series	General public	Health center talks
May	Mosque & Community Center Outreach	Community members	Outreach sessions
June	Hypertension Awareness Drive	High-risk groups	Screening + talk
July	Mall / Public Space BE-FAST Campaign	General public	Public campaign
August	Caregiver Education Session	Caregivers of stroke survivors	Workshop
September	Media Engagement Month	General public	TV / radio / press
October	World Stroke Day Campaign (29 Oct)	General public	Multi-channel campaign
November	Diabetes & Stroke Risk Awareness	High-risk groups	Screening + talk
December	Annual Review & Materials Update	Internal / IPC	Committee review

7. Educational Materials and Resources

- BE-FAST brochures and posters (Arabic / English) displayed across outpatient and public areas.
- BE-FAST wallet cards distributed at community events.
- Digital assets adapted from AHA/ASA public education resources for local social media use.
- Short educational videos for waiting-area screens and social media.
- Risk-factor self-assessment handouts for screening events.

8. Community Partnerships

- Saudi Red Crescent Authority / EMS for joint public messaging on emergency activation.
- Ministry of Education / local schools for student awareness sessions.
- Local mosques and community centers for outreach events.
- Local media outlets for campaign amplification.
- Primary healthcare centers within the Jazan Health Cluster network.

9. Program Evaluation

The effectiveness of the public education plan will be evaluated using:

- Number of community outreach events and screening sessions conducted per year.
- Total public reach (attendance, campaign impressions, media coverage).
- Pre/post-session BE-FAST awareness assessment scores at community events.
- Trends in community-reported onset-to-door time for stroke presentations.
- Number of individuals screened and referred for risk-factor follow-up.

Results will be reviewed annually by the Stroke Interprofessional Committee and used to refine outreach priorities and materials for the following year.

10. References

- American Heart Association / American Stroke Association Target: Stroke Best Practice Strategies.
- AHA/ASA public stroke-awareness education resources (BE-FAST campaign materials).
- AHA/ASA Guideline for the Early Management of Patients with Acute Ischemic Stroke.
- World Stroke Organization — World Stroke Day campaign materials.

11. Approval

Name	Title	Signature	Date
	Stroke Program Director		
	Stroke Program Coordinator		
	Chair, Stroke Interprofessional Committee		
	Community Health / Public Relations Lead		